TRENDS IN TUBERCULOSIS

In 1964 the number of new active tuberculosis cases and the case rate for the United States decreased markedly. The States reported 50,874 new active tuberculosis cases in 1964, or 3,168 fewer than the 54,042 that were recorded in the previous year. The relative magnitude of this one year reduction was proportionately as great as the average annual change for the decade prior to 1962. The new case rate for the United States was 26.6 per 100,000 population in 1964 as compared with 28.7 in 1963.

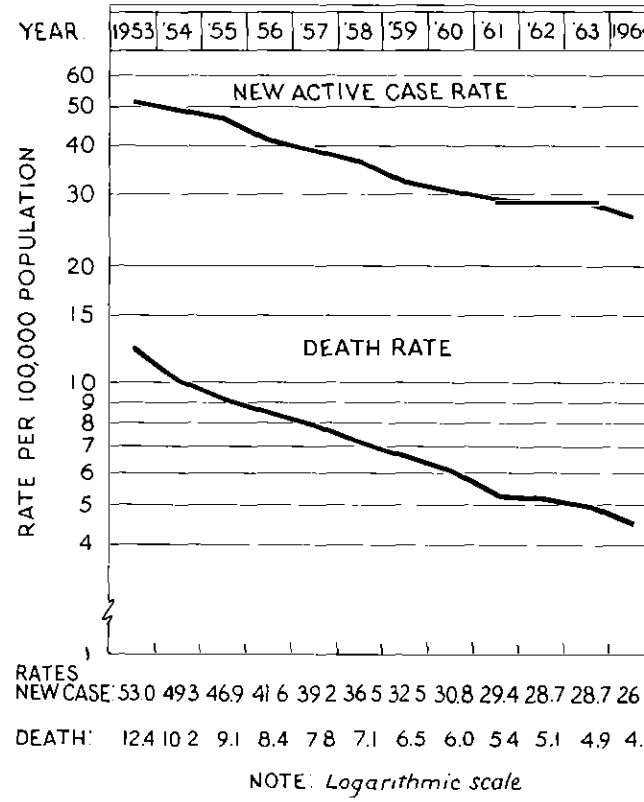
During the last two years there has been a marked improvement in the reporting of new cases. Most of the 1961 Public Health Service recommendations on the reporting of tuberculosis have been adopted by the States. All counties and major cities are being reported upon, even though in a few instances details are not available. Minor differences in interpreting the recommendations, however, still exist. These are being clarified to bring about uniform reporting practices throughout the entire country.

Seventeen States and the District of Columbia reported a combined increase of 774 new active cases in 1964, while thirty-three States reported a total of 3,942 fewer cases than in 1963. The greatest single change occurred in New York State where 834 fewer cases were recorded.

For the country as a whole, even though the total number of new cases decreased, the percent distribution of new active tuberculosis cases by age, sex, and race in 1964 was almost identical with 1963.

Totals for the United States are a composite and, therefore, new case rates for the country represent an average. As has been pointed out in earlier reports the nationwide figures are points of reference, and not standards by which the tuberculosis situation in a particular community can be assessed. An evaluation of tuberculosis trends in a specific State or city, is more realistic if the local data are compared with places having similar

FIGURE 1. *New active tuberculosis case rate and death rate, United States, 1953–1964*



demographic characteristics. In addition, the interpretation of tuberculosis trends for any particular community should also include the various elements of tuberculosis control that influence morbidity and mortality in that area.

The national statistics that follow contain some of the numerical components of tuberculosis epidemiology that should be taken into account when interpreting the trend of tuberculosis morbidity in any community. A number of studies have shown that factors that increase the risk of developing active disease are closely associated with the age, sex, race, and socio-economic status of a